

NAVIGATE Paper Outline With Embedded Quotes and Citations

Working title: Evaluating NAVIGATE as a Coordinated Specialty Care Intervention for First-Episode Psychosis
Target length: 5-7 pages of body text, plus references APA style: 7th edition Purpose of this version: This outline keeps the original paper structure, but now adds direct quotes, source notes, and citation reminders under each section so it can double as a practical writing guide.

One-sentence thesis

NAVIGATE is a strong, recovery-oriented coordinated specialty care (CSC) intervention for first-episode psychosis with meaningful support from a major U.S. randomized trial and implementation/fidelity research, but under a conservative Youngstrom et al. (2021) evidence-base framework, the full NAVIGATE package is still best classified as Possibly Efficacious rather than Well-Established because the model does not yet appear to have multiple independent replications of the full program.

Recommended Paper Structure by Rubric Category

I. Introduction + Thesis + Roadmap (about 0.5 page)

Rubric fit: Writing; introduces topic, intervention, and argument clearly Suggested topic sentence: First-episode psychosis requires early, coordinated, recovery-focused treatment, and NAVIGATE has become one of the best-known U.S. models designed to meet that need.

Bullet points to include

- Briefly define first-episode psychosis (FEP).
- Explain why early intervention matters: psychosis disrupts school, work, relationships, and functioning.
- Introduce NAVIGATE as a U.S.-developed coordinated specialty care model created in the NIMH RAISE Early Treatment Program.
- Preview the paper's argument: NAVIGATE is clinically meaningful and evidence-informed, but the full package still has limited replication.

Evidence/citation notes

- Cite Penn et al. (2015) for the program rationale and U.S. development context.
- Cite Kane et al. (2016) for the major outcome trial.
- Cite Youngstrom et al. (2021) when introducing the evidence-base framework.

Direct quotes you can use

- Penn et al. (2015), abstract: "NAVIGATE is a team-based, multi-component treatment program designed to be implemented in routine mental health treatment settings and aimed at guiding people with a first episode of psychosis (and their families) towards psychological and functional health."
- Good use: opening description of what NAVIGATE is.

- Kane et al. (2016), abstract/conclusions: “Comprehensive care for first-episode psychosis can be implemented in U.S. community clinics and improves functional and clinical outcomes.”
- Good use: introduction or conclusion sentence showing practical significance.
- Kane et al. (2016), abstract/conclusions: “Effects are more pronounced for those with shorter duration of untreated psychosis.”
- Good use: early-intervention rationale.

Possible thesis paragraph language

- NAVIGATE appears to improve quality of life, psychopathology, and participation in work or school for people with first-episode psychosis (Kane et al., 2016).
- The model is also manualized and implementable with acceptable fidelity in community settings (Mueser et al., 2019).
- However, the full intervention package still appears to rely heavily on one primary line of research, so a conservative evidence-base classification is more defensible than an inflated one.

II. Summary of the Intervention (about 1.25-1.5 pages)

Rubric fit: Summary of the Intervention (20 points) Suggested topic sentence: NAVIGATE is a multidisciplinary, manualized treatment program designed to promote symptom improvement, recovery, and functional progress after a first episode of psychosis.

A. Presenting problem targeted

- NAVIGATE targets first-episode nonaffective psychosis / schizophrenia-spectrum disorders in the early phase of treatment (Penn et al., 2015; Kane et al., 2016).
- Main clinical problems addressed:
 - positive symptoms;
 - distress and impairment;
 - reduced quality of life;
 - poor role functioning;
 - disruption in school/employment;
 - family confusion and burden after the first episode.

Direct quotes

- Penn et al. (2015): “The focus of NAVIGATE is on helping individuals who are experiencing their first episode of psychosis, regardless of the duration of symptoms, and have received no or limited antipsychotic medication, or who have recently received treatment for a first episode, and are therefore early in their treatment of psychosis.”
- Kane et al. (2016), abstract/method: Participants had “schizophrenia and related disorders and ≤ 6 months of antipsychotic treatment.”

Citation reminders

- Use Penn et al. (2015) when describing the target condition and treatment purpose.
- Use Kane et al. (2016) when describing how the study operationalized the target sample.

B. Target population

- Typical NAVIGATE participants were adolescents and adults in the early course of psychosis.
- The program primarily targeted individuals 15-35 years, but could include some up to age 40 (Penn et al., 2015).
- Family involvement is encouraged when available and desired.
- The model was designed for community mental health settings, not just elite research clinics.

Direct quotes

- Penn et al. (2015): "The program primarily targets individuals, aged 15 to 35 years, when schizophrenia-spectrum disorders are most likely to develop..."
- Penn et al. (2015): "NAVIGATE can also accommodate older individuals (up to age 40) who have also recently developed a first episode of psychosis."
- Penn et al. (2015): "The NAVIGATE program was designed to be implemented in typical non-academic U.S. mental health care settings..."

Citation reminders

- Cite Penn et al. (2015) for age range and intended setting.
- Cite Kane et al. (2016) for the actual study sample mean age (23) if needed.

C. Core components of NAVIGATE

Suggested topic sentence: NAVIGATE combines medication, psychotherapy, family work, and educational/vocational support within one coordinated, shared-decision-making model.

1. Personalized Medication Management (PMM)

- Evidence-informed medication prescribing.
- Emphasis on low effective doses, systematic monitoring, and tolerability (Penn et al., 2015).
- Shared decision-making is central, not optional.

Direct quotes

- Penn et al. (2015): "Most comprehensive programs provide individualized treatment planning and low dose, evidence-based pharmacotherapy that strives to minimize side effects and non-adherence."
- Penn et al. (2015): The medication prescriber provides "individualized medication treatment, including systematic monitoring of signs, symptoms, and side effects, and guideline-based pharmacological treatment."

Citation reminders

- Penn et al. (2015) for PMM description.
- Mueser et al. (2019) when noting PMM was one of the fidelity-rated components.

2. Individual Resiliency Training (IRT)

- Individual therapy focused on recovery goals, wellness, psychoeducation, coping, and illness self-management (Penn et al., 2015).
- Draws on CBT and motivational approaches.

Direct quotes

- Penn et al. (2015): IRT is “a psychotherapeutic approach aimed at helping clients set personal goals, enhance wellness and personal resiliency, learn about psychosis and its treatment, improve illness self-management, and progress towards personal goals.”
- Penn et al. (2015): “Psychotherapeutic interventions based on cognitive behavioral therapy and motivational interviewing [are] offered to all clients.”

Citation reminders

- Penn et al. (2015) for IRT content.
- Mueser et al. (2019) when discussing IRT as a fidelity-assessed component.

3. Family Education Program (FEP)

- Psychoeducation and collaborative work with family/supports.
- Helps families understand psychosis, reduce stress, and support recovery goals (Penn et al., 2015).

Direct quotes

- Penn et al. (2015): The Family Education Program is “aimed at developing a collaborative relationship with family members, educating them about psychosis and its treatment, and enlisting their support for the client’s involvement in treatment and pursuit of personal goals.”
- Penn et al. (2015): “Most persons who develop a first episode psychosis are living with or in regular contact with family members, so family intervention is also a standard.”

Citation reminders

- Penn et al. (2015) for family intervention rationale and description.
- Mueser et al. (2019) for fidelity evidence related to family education.

4. Supported Employment and Education (SEE)

- Helps clients pursue work and school goals quickly and practically.
- Important because recovery in FEP includes real-life role functioning, not just symptom reduction.

Direct quotes

- Penn et al. (2015): The SEE specialist “helps clients identify or develop, and pursue personally meaningful goals related to education and competitive employment.”
- Kane et al. (2016), abstract/results: NAVIGATE participants “experienced greater involvement in work and school compared with 181 participants in community care.”

Citation reminders

- Penn et al. (2015) for description of SEE.
- Kane et al. (2016) for outcome support related to work/school involvement.

5. Team structure / overall organization

- NAVIGATE is team-based and coordinated across providers.
- Shared decision-making, strengths, and resiliency are model-wide principles, not just single-session techniques.

Direct quotes

- Penn et al. (2015): “Staffing for the NAVIGATE program is multidisciplinary, with team members working together to implement treatment and each person providing a specific intervention.”
- Penn et al. (2015): “Shared decision-making recognizes that all people have the right to make decisions about their own treatment, based on their own preferences and goals.”
- Penn et al. (2015): “Treatment decisions are made by the client and clinician in partnership together...”

Citation reminders

- Penn et al. (2015) for team structure and conceptual foundations.
- Mueser et al. (2019) for organization/staffing fidelity.

D. Measures or indicators of treatment progress

- Quality of life was the primary outcome in the RAISE trial (Kane et al., 2016).
- Other indicators included psychopathology, work/school participation, retention, and possibly hospitalization.
- Clinically, treatment progress can also be tracked through symptom stabilization, family collaboration, and movement toward personal goals.

Direct quotes

- Kane et al. (2016), abstract/method: “The primary outcome was the total score of the Heinrichs-Carpenter Quality of Life Scale...”
- Kane et al. (2016), abstract/results: NAVIGATE recipients “remained in treatment longer, experienced greater improvement in quality of life and psychopathology, and experienced greater involvement in work and school...”

Citation reminders

- Kane et al. (2016) for primary and secondary outcome indicators.

E. Training/experience needed to deliver NAVIGATE

Suggested topic sentence: NAVIGATE was designed for real-world settings, but it still requires manuals, structured training, role-specific competencies, and fidelity monitoring.

- NAVIGATE is not casual supportive counseling; it is a manualized multicomponent model.
- The implementation study covered 17 sites and 134 practitioners (Mueser et al., 2019).
- Clinicians need role-specific competency for medication, IRT, family work, SEE, and team leadership.
- A strong point for critique: the model is transportable, but it still depends on organizational support.

Direct quotes

- Penn et al. (2015): "NAVIGATE is standardized in six manuals..."
- Mueser et al. (2019), abstract: This article described implementation at "the 17 sites (including 134 practitioners) randomized to provide it..."
- Mueser et al. (2019), abstract: "Fidelity was evaluated to five different components of the program, all of which were standardized in manuals in advance of implementation."

Citation reminders

- Penn et al. (2015) for manuals and team roles.
- Mueser et al. (2019) for training/fidelity evidence.

Optional critique sentence for this section

- A major strength of NAVIGATE is that it targets both symptom reduction and functional recovery, but the multicomponent design also makes it harder to identify which ingredients are most responsible for change.

III. Summary and Critique of the Literature (about 1.5-2 pages)

Rubric fit: Summary and Critique of the Literature (30 points) Suggested topic sentence: The literature on NAVIGATE supports its clinical value and feasibility, but the evidence base for the full package remains narrower than its reputation might suggest.

Article 1: Kane et al. (2016) / RAISE Early Treatment Program outcomes trial

Citation: Kane, J. M., Robinson, D. G., Schooler, N. R., et al. (2016). Comprehensive versus usual community care for first-episode psychosis: 2-year outcomes from the NIMH RAISE early treatment program. *American Journal of Psychiatry*, 173(4), 362-372. <https://doi.org/10.1176/appi.ajp.2015.15050632>

Summary points

- Cluster-randomized trial at 34 clinics in 21 states (Kane et al., 2016).

- Included 404 participants with schizophrenia-spectrum disorders and limited antipsychotic exposure.
- Compared NAVIGATE with usual community care over at least 2 years.
- Main findings:
 - better quality of life;
 - better psychopathology/symptom outcomes;
 - greater involvement in work and school;
 - longer retention;
 - stronger benefits with shorter duration of untreated psychosis.

Direct quotes

- Kane et al. (2016), abstract/objective: “The primary aim of this study was to compare the impact of NAVIGATE...with community care on quality of life.”
- Kane et al. (2016), abstract/results: “The 223 recipients of NAVIGATE remained in treatment longer, experienced greater improvement in quality of life and psychopathology, and experienced greater involvement in work and school compared with 181 participants in community care.”
- Kane et al. (2016), abstract/results: “NAVIGATE participants with duration of untreated psychosis of <74 weeks had greater improvement in quality of life and psychopathology...”
- Kane et al. (2016), abstract/conclusions: “Comprehensive care for first-episode psychosis can be implemented in U.S. community clinics and improves functional and clinical outcomes.”

Critique points

- Strength: Large multisite real-world U.S. design improves ecological validity.
- Strength: Comparison against usual community care makes the results clinically meaningful.
- Limitation: “Usual care” is a variable comparison condition rather than a highly standardized active comparator.
- Limitation: The study tests a package, not isolated components.
- Limitation: One major positive trial is important, but one major trial is still not the same thing as broad replication.
- Nuance: Hospitalization differences were not significant, which helps keep the discussion balanced.

Helpful synthesis line

- This article is the strongest empirical reason to argue that NAVIGATE is more than just a nice theory; it produced meaningful functional and clinical gains in ordinary U.S. clinics.

Article 2: Mueser et al. (2019) / implementation and fidelity study

Citation: Mueser, K. T., Meyer-Kalos, P. S., Glynn, S. M., et al. (2019). Implementation and fidelity assessment of the NAVIGATE treatment program for first episode psychosis in a multi-site study. *Schizophrenia Research*, 204, 271-281. <https://doi.org/10.1016/j.schres.2018.08.015>

Summary points

- Described training and implementation at the 17 NAVIGATE sites.
- Included 134 practitioners (Mueser et al., 2019).
- Assessed fidelity for IRT, FEP, SEE, PMM, and team organization.
- Found acceptable or better fidelity across sites, with all 17 reaching acceptable overall fidelity.

Direct quotes

- Mueser et al. (2019), abstract: “The NAVIGATE program was developed for the Recovery After Initial Schizophrenia Episode-Early Treatment Program (RAISE-ETP) study...”
- Mueser et al. (2019), abstract: “Most of the sites demonstrated acceptable or higher levels of fidelity...”
- Mueser et al. (2019), abstract: “All 17 sites demonstrat[ed] at least acceptable overall fidelity to the NAVIGATE program.”
- Mueser et al. (2019), abstract: “The results indicate that the NAVIGATE program can be implemented with good fidelity to the treatment model in a diverse array of community mental health care settings...”

Critique points

- Strength: This paper addresses whether the intervention can actually be delivered as intended in routine care.
- Strength: Fidelity data make the outcome literature more believable because the model was operationalized rather than vaguely defined.
- Limitation: Fidelity does not equal efficacy.
- Limitation: The implementation literature is still closely tied to the original research network.
- Limitation worth mentioning: The paper discloses later consultation/training relationships; this does not invalidate the study, but it should be acknowledged.

Helpful synthesis line

- This article strengthens the case that NAVIGATE is transportable and structured, but it does not replace the need for multiple independent outcome trials.

Article 3: Penn et al. (2015) / rationale and psychosocial description

Citation: Penn, D. L., Addington, J., Brunette, M. F., et al. (2015). The NAVIGATE program for first-episode psychosis: Rationale, overview, and description of psychosocial components. *Psychiatric Services*, 66(7), 680-690. <https://doi.org/10.1176/appi.ps.201400413>

How to use this source

- Use this paper mostly for:
- program rationale,
- conceptual foundations,
- team roles,

- component descriptions,
- shared decision-making and recovery orientation.
- It is not the strongest source for empirical outcomes, so keep it secondary when discussing efficacy.

Direct quotes

- Penn et al. (2015), abstract: “The core services provided in the NAVIGATE program include the Family Education Program, Individual Resiliency Training, Supported Employment and Education, and Individualized Medication Treatment.”
- Penn et al. (2015), abstract: “NAVIGATE embraces a shared decision-making approach with a focus on strengths and resiliency...”
- Penn et al. (2015): “The goal of the NAVIGATE program is recovery, defined by each individual in their own terms...”

Critique point

- This article is valuable for understanding what NAVIGATE is supposed to be, but it should not be overused as proof that NAVIGATE works.

Mini-synthesis sentence for the end of this section

- Taken together, the literature shows that NAVIGATE is manualized, feasible, and associated with better outcomes than usual care, but the evidence base for the full branded package remains concentrated within one primary research program.

IV. JCCAP Evidence-Base Evaluation Using Youngstrom et al. (2021) (about 1.25-1.5 pages)

Rubric fit: Intervention review using evidence-base criteria (35 points) Suggested topic sentence: When NAVIGATE is evaluated conservatively against Youngstrom et al. (2021), it looks clearly promising and clinically meaningful, but not yet replicated enough to justify a “well-established” label.

Recommended designation: Possibly Efficacious

Why this level is the safest argument

- NAVIGATE has one major randomized controlled trial with positive findings on multiple meaningful outcomes (Kane et al., 2016).
- It also has supportive implementation/fidelity evidence showing the model can be delivered with acceptable adherence (Mueser et al., 2019).
- The program is clearly manualized and conceptually coherent (Penn et al., 2015).
- However, the full package does not yet appear to have multiple independent randomized replications across separate investigative teams.

How to connect this to Youngstrom/JCCAP categories

- Well-Established: usually requires multiple rigorous studies and independent replication.
- Probably Efficacious: stronger than preliminary support, but still missing some criterion.
- Possibly Efficacious: positive evidence exists, but the replication base is still limited.
- Experimental/Questionable Efficacy: too weak for NAVIGATE, because NAVIGATE has at least one major positive trial and fidelity support.

Core rationale paragraph points

- NAVIGATE should not be labeled Experimental or Questionable because it has clear empirical support.
- A Probably Efficacious argument is not absurd, especially if someone emphasizes the size and real-world relevance of RAISE-ETP.
- Still, Possibly Efficacious is easier to defend because:
 - the full package has limited direct replication;
 - the literature is still closely tied to the originating research network;
 - there is limited evidence from independent head-to-head trials against other strong CSC models.

Direct quotes you can use in this evaluative section

- Kane et al. (2016): "Comprehensive care for first-episode psychosis can be implemented in U.S. community clinics and improves functional and clinical outcomes."
- Use this to support that NAVIGATE clearly has real evidence of benefit.
- Mueser et al. (2019): "The results indicate that the NAVIGATE program can be implemented with good fidelity..."
- Use this to support that NAVIGATE is manualized and transportable, not just theoretical.

Strong sentence to use in the paper

- Although NAVIGATE has stronger support than many emerging interventions for first-episode psychosis, the complete treatment package does not yet appear to satisfy the highest JCCAP-style evidence standards that usually require repeated, independent replication; therefore, the most conservative classification is Possibly Efficacious.

What research is needed for NAVIGATE to become Well-Established?

- Additional independent randomized controlled trials of the full NAVIGATE model.
- Replication outside the original RAISE/NAVIGATE network.
- Comparisons with other strong CSC or early psychosis programs, not only variable usual care.
- More evidence in racially/ethnically diverse, rural, and under-resourced settings.
- Longer-term follow-up on relapse, functioning, hospitalization, and sustained school/work participation.
- Component/dismantling studies clarifying which pieces are essential.
- Dissemination and cost/scalability research.

Optional nuance sentence

- It is possible that some individual NAVIGATE components draw from stronger prior evidence bases than the full combined NAVIGATE package itself, which is another reason to be careful not to overclaim the status of the branded intervention.

V. Conclusion (about 0.5 page)

Rubric fit: Writing and synthesis Suggested topic sentence: Overall, NAVIGATE stands out as a clinically meaningful and practical intervention for first-episode psychosis, even if its formal evidence-base designation should remain conservative.

Bullet points to include

- Re-state the thesis in fresh wording.
- Emphasize practical strengths:
 - recovery-oriented;
 - team-based;
 - workable in community settings;
 - addresses both symptoms and functioning.
- Re-state the evidence judgment: likely Possibly Efficacious under a conservative Youngstrom framework.
- End with a forward-looking statement about independent replication.

Direct quotes

- Kane et al. (2016): "Comprehensive care for first-episode psychosis can be implemented in U.S. community clinics and improves functional and clinical outcomes."
- Penn et al. (2015), abstract: NAVIGATE "has the potential to fill an important gap in the U.S. healthcare system by providing a comprehensive intervention specially designed to meet the unique treatment needs of persons recovering from a first episode of psychosis."

Possible final sentence

- In short, NAVIGATE is a serious, evidence-informed intervention with meaningful support, but the literature is not yet broad enough to justify calling the full program well-established.

Optional Background Source: Is PMC6526799 Actually Relevant?

Article

Driver, D. I., Thomas, S., Gogtay, N., & Rapoport, J. L. (2019). A review of childhood-onset schizophrenia. *Child and Adolescent Psychiatric Clinics of North America*, 29(1), 71-85.
<https://pmc.ncbi.nlm.nih.gov/articles/PMC6526799/>

Relevance judgment

This is only tangentially relevant to a NAVIGATE/first-episode psychosis intervention paper. It should be treated as optional background, not a core NAVIGATE source.

Why it is not a core source

- It is a broad review of childhood-onset and early-onset schizophrenia, not a paper about NAVIGATE itself.
- It does not evaluate the NAVIGATE intervention as a treatment package.
- It may help with general background on early psychosis, developmental context, or the importance of early treatment, but it should not anchor the intervention review or evidence-base argument.

Quote you could use only if needed for broad background

- Driver et al. (2019): "Although there is no cure for childhood-onset schizophrenia or even a single medication that is most efficacious, there is a consensus that early treatment is vital..."
- Use only for a general statement about the importance of early treatment, not as proof that NAVIGATE works.

Caution

- If space is tight, skip this source.
- If included, label it clearly as general background on early psychosis/schizophrenia, not as one of the core NAVIGATE efficacy studies.

Suggested Page Allocation for a 5-7 Page Paper

- Introduction: 0.5 page
- Summary of intervention: 1.25-1.5 pages
- Summary and critique of literature: 1.5-2 pages
- JCCAP evidence-base evaluation: 1.25-1.5 pages
- Conclusion: 0.5 page
- References: separate final page/section

This should land around 5-6 pages of body text, which fits the assignment safely.

Cleaner APA-Style References (Draft)

References

- Driver, D. I., Thomas, S., Gogtay, N., & Rapoport, J. L. (2019). A review of childhood-onset schizophrenia. *Child and Adolescent Psychiatric Clinics of North America*, 29(1), 71-85. <https://pmc.ncbi.nlm.nih.gov/articles/PMC6526799/>
- Kane, J. M., Robinson, D. G., Schooler, N. R., Mueser, K. T., Penn, D. L., Rosenheck, R. A., Addington, J., Brunette, M. F., Correll, C. U., Estroff, S. E., Marcy, P., Robinson, J., Meyer-Kalos, P. S., Glynn, S. M., Lynde, D. W., Pipes, R., Kurian, B. T., Miller, A. L., Azrin, S. T., Goldstein, A. B., Severe, J. B., Lin, H., Sint, K. J., John, M., & Heinssen, R. K. (2016). Comprehensive versus usual community care for first-episode psychosis: 2-year outcomes from the NIMH RAISE early treatment program. *American Journal of Psychiatry*, 173(4), 362-372. <https://doi.org/10.1176/appi.ajp.2015.15050632>
- Mueser, K. T., Meyer-Kalos, P. S., Glynn, S. M., Lynde, D. W., Robinson, D. G., Gingerich, S., Penn, D. L., Cather, C., Gottlieb, J. D., Marcy, P., Wiseman, J. L., Potretzke, S., Brunette, M. F., Schooler, N. R., Addington, J., Rosenheck, R. A., Estroff, S. E., & Kane, J. M. (2019). Implementation and fidelity assessment of the NAVIGATE treatment program for first episode psychosis in a multi-site study. *Schizophrenia Research*, 204, 271-281. <https://doi.org/10.1016/j.schres.2018.08.015>
- Penn, D. L., Addington, J., Brunette, M. F., Gingerich, S., Glynn, S. M., Lynde, D. W., Gottlieb, J. D., Meyer-Kalos, P. S., McGurk, S. R., Cather, C., Saade, S., Robinson, D. G., Schooler, N. R., Rosenheck, R. A., & Kane, J. M. (2015). The NAVIGATE program for first-episode psychosis: Rationale, overview, and description of psychosocial components. *Psychiatric Services*, 66(7), 680-690. <https://doi.org/10.1176/appi.ps.201400413>
- Youngstrom, E. A., Choukas-Bradley, S., Calhoun, C. D., & Jensen-Doss, A. (2021). Clinical guide to the evidence-based update series: Standards, issues, and applications. *Journal of Clinical Child & Adolescent Psychology*, 50(5), 565-582.

Important verification note: I did not have a reliable full-text pull of the Youngstrom article in this session, so the Youngstrom reference should be cross-checked against the course-assigned source before final submission. Use the exact course citation if your instructor assigned a different Youngstrom et al. (2021) reading or excerpt.

Quick Writing Tips

- Keep tying each section back to the thesis so this does not turn into a random article summary.
- Use Kane et al. (2016) as the main efficacy paper.
- Use Mueser et al. (2019) to support feasibility/fidelity, not as a substitute for outcomes.
- Use Penn et al. (2015) to explain what NAVIGATE is and why it was designed that way.
- Treat Driver et al. (2019) / PMC6526799 as optional background only.
- Do not overclaim that NAVIGATE is Well-Established unless you find multiple independent full-package replications.
- If you need to cut for length, trim some intervention description before cutting the evidence-base analysis.